

Solitary Confinement

– History, Practice, and Human Rights Standards¹

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The history of solitary confinement

In Europe and North America between the 18th and 19th centuries, the approach to criminal punishment shifted significantly. A system of religiously motivated corporal and public punishment was very gradually substituted by a modern bureaucratic and rational system, which was based on secular laws and had the use of imprisonment as a central feature. As part of this process the practice of imprisonment was itself reformed as the ideology of the modern penitentiary took hold from the 1770s to the 1850s. A belief in the ability to rehabilitate criminals through the use of isolation regimes became a cornerstone of this modern prison system.

This process was fuelled by the enlightenment which resulted in law reforms and the formation of new legal principles now associated with the concept of the 'Rechtsstaat' and that of the 'Rule of law' — essential parts of any democracy. Towards the end of the 18th century — especially from the 1770s onwards — local experiments with new prison structures, systems and regimes took place in various nations and the ideology of the modern penitentiary began to take shape. The principle of rehabilitating prisoners through a system of isolation and moral reform was associated with some of these experiments in, for example, England and Pennsylvania in the US. But it was not until the construction of the so-called Auburn and Pennsylvania prison models in the US during the 1820s that the principle of rehabilitation of criminals through the use of isolation became an ideology influencing prison construction and prison regimes throughout the Western world.

The Auburn system permitted inmates to work together during the day, but under a regime of total silence where no communication was permitted. In Pennsylvania-model institutions, on the other hand, there was no compromise with the ideal of isolation, and prisoners spent all their time in their cells where they also did their work. Here the inmate was supposed to turn his thoughts inward and meet both God and his inner self, and through such a personal religious experience, to return to society as a morally cleansed Christian citizen. In other words, Pennsylvania-model prisons created a system of large scale solitary confinement of sentenced prisoners which was applied in prisons in many states in different parts of the world.³

In the US the Auburn model became the most popular, but in Europe the Pennsylvania system was typically favoured. Pennsylvania-model prisons — or variants resembling them — were constructed in many nations in different parts of the world, including England, France, Germany, Belgium, Holland, Norway, Sweden, Denmark, Chile, New Zealand, and USA.⁴ The exact construction, regime, and routines in these institutions varied from country to country and from prison to prison, but they all shared common traits, often to a remarkable degree. In general terms the regime in Pennsylvania model institutions consisted of around 23 hours in cell, where the prisoners worked, slept, ate, and washed. Out of cell time typically consisted of one half hour to one hour yard time, normally in solitary confinement and possibly — depending on the prison — of a weekly visit to the prison church or perhaps a prison school. Many of these prison churches and sometimes also the prison schools were constructed panoptically with isolation booths for individual prisoners, maintaining surveillance and solitary confinement even at these times. The wearing of a prison mask was also a regular feature of Pennsylvania model prisons and inmates were required

1. Parts of this article are based on my previous work including: Peter Scharff Smith (2006) 'The effects of solitary confinement on prison inmates: a brief history and review of the literature', in *Crime and Justice*, vol. 34, University of Chicago Press, pp. 441-528, 2006; Peter Scharff Smith (2008) 'Solitary confinement: an introduction to the Istanbul Statement on the Use and Effects of Solitary Confinement', in *Journal on Rehabilitation of Torture Victims and Prevention of Torture*, vol. 18, No. 1, pp. 56-62, 2008; Peter Scharff Smith (2004) 'Isolation and Mental Illness in Vridsløselille 1859-1873 — A New Perspective on the Breakthrough of the Modern Penitentiary' in *Scandinavian Journal of History*. Vol. 29, no. 1, pp. 1-25, 2004 (A).
2. The author is grateful to Jonathan Beynon for commenting on an earlier draft.
3. See for example Michel Foucault (1995) *Discipline & Punish*, Vintage Books 1995, Michael Ignatieff (1978) *A Just Measure of Pain*, Macmillan 1978, Mark Colvin (1997) *Penitentiaries, Reformatories, and Chain Gangs. Social Theory and the History of Punishment in Nineteenth-Century America*, New York: St. Martin's Press 1997, and Peter Scharff Smith (2004) 'A Religious Technology of the Self. Rationality and Religion in the Rise of the Modern Penitentiary' in *Punishment and Society* no. 2, 2004 (B).
4. See for example Norman Johnston (2000) 'Forms of Constraint. A History of Prison Architecture', Chicago: University of Illinois Press, 2000, chapter 4-6; Ricardo Salvatore and Carlos Aguirre (ed.) (1996) 'The Birth of the Penitentiary in Latin America', Austin: University of Texas Press, 1996, chapter 1.

to put this on while moving around the prison so they could not see the face of another inmate, or be seen. Solitary confinement was thereby almost complete. The only exceptions were the visits of 'moral agents' who were supposed to further the reform process.⁵ This typically meant that the prison governor, the prison chaplain and various other educated staff engaged individual prisoners in private conversations in their cells. In reality, these conversations had a limited impact due to the high ratio of prisoners to 'moral agents' in a modern penitentiary. In a Danish Pennsylvania model prison the governor calculated that each inmate could — as a maximum — expect a daily rate of six minutes personal conversation.⁶ The staff simply did not have sufficient time.

As described by a Danish prisoner at the time, one was 'instantly overpowered' by a 'depressing' and 'poignant solitude' that went against the natural desire of 'both men of nature and men of culture' for a social life. As one who had experienced it himself, he knew 'what it meant to measure out the cell with tiny footsteps hour after hour, while over and over again a perpetual emptiness grinds away and throws the prisoner into a condition which borders on insanity'.⁷

Partly due to the pronounced negative health effects associated with the prolonged use of solitary confinement, documented at the time, the Pennsylvania model passed gradually out of use — but at very different points in time in different countries. The use of large scale Pennsylvania model solitary confinement was abandoned relatively quickly in several US states — around the middle of the 19th century — but continued in several places in Europe. In Holland, Belgium, Sweden, Norway, and Denmark, the model continued well into the 20th century.⁸

After the Second World War a new use for solitary confinement emerged in connection with stories about

Chinese 'brainwashing' of prisoners of war (POWs) in the Korean War. Cases of 'thought reform' were apparently achieved through the use of techniques that involved social isolation. Later research confirmed that solitary confinement was used as a coercive measure in some Communist countries during the cold war. In the USSR, for example, the KGB used it pre-trial with so-called 'enemies of the state' as a coercive tool in an interrogation — and confession — process.⁹

These practices caught the attention of psychologists and psychiatrists who during the 1960s and 1970s conducted countless experiments in sensory deprivation that described how isolation could have dramatic impact on individuals. The CIA was very interested in both the communist prison practices and the scientific research as becomes apparent to anyone who cares to study the CIA's manual on *Counterintelligence interrogation* from 1963 as well as their later *Human Resource Exploitation Training Manual* from 1983. The former describes how solitary confinement can be used coercively together with focused interrogations. According to the manual the interrogator can exploit the otherwise isolated detainee's intense need for social

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contact and sensory input: 'As the interrogator becomes linked in the subject's mind with the reward of lessened anxiety, human contact, and meaningful activity, and thus with providing relief for growing discomfort, the questioner assumes a benevolent role ... the regressed subject [will] view the interrogator as a father-figure. The result, normally, is a strengthening of the subject's tendencies toward compliance'.¹⁰ It is now well known how US personnel have used solitary confinement in similar ways at the Guantanamo Bay detention camp, as well as in facilities in Iraq and Afghanistan.

In addition to the above, solitary confinement has continued to be used in most prison systems in some

5. See, for example, Smith 2004 (A).

6. Peter Scharff Smith (2003) 'Moralske hospitaler — Det moderne fængselsvæsens gennembrud internationalt og i Danmark ca. 1770-1870', *Forum*, 2003, p. 234.

7. Johnsen (1929), quoted from Peter Scharff Smith "'Degenerate Criminals'. Mental Health and Psychiatric Studies of Danish Prisoners in Solitary Confinement, 1870—1920", in *Criminal Justice and Behavior*, Vol. 35, No. 8, 2008, p. 1050.

8. Smith 2006, p. 467.

9. Hinkle LE and Wolff HG. (1956) Communist interrogation and indoctrination of 'enemies of the state'. *Arch. Neurol. Psychiatry* 1956;76:115-74.

10. KUBARK Counterintelligence Interrogation 1963, p. 90. See www.gwu.edu/~nsarchiv/NSAEBB/NSAEBB122/index.htm#kubark. (assessed September 2008).

form — for example as a punishment for infraction of prison rules — but on a much smaller scale than was the case with the Pennsylvania prison model. Many of these practices, however, remain in operation.

Contemporary solitary confinement practices

The latest report to the UN General Assembly from the UN Special Rapporteur on Torture, Manfred Nowak, has a whole chapter devoted to the issue of solitary confinement. Here he briefly touches upon some of the current isolation practices in different prisons in different parts of the world: as disciplinary punishment, as a pre-trial measure, as a regime on death row, as a detention and interrogation tool used to either 'train' individuals or elicit information from them, and finally as a purely punitive measure used to 'impose additional punishment'.¹¹ In the following I will provide examples of these and other topical solitary confinement practices.

First and foremost, most prison systems feature solitary confinement among their repertoire of disciplinary punishments for prisoners. There are countless variations in this regard but typically, although not always, such punishment will last for a limited number of days or perhaps weeks. In Denmark, for example, the maximum duration of placement in isolation as a punishment for violating prison rules, is four weeks.¹² Sometimes, in complete contradiction to international standards, restraints are also applied on such prisoners. In a prison in Nigeria the UN Special Rapporteur on Torture for example encountered 'a teenage boy suffering from severe mental illness, locked in a punishment cell with his feet shackled'.¹³

In addition to the use of isolation as a response to violations of prison rules there are usually a small

number of inmates in any prison system who the authorities find very difficult to handle under normal conditions. Special regimes involving prolonged solitary confinement are often used for such prisoners. Such a practice is technically speaking not punishment, and is typically referred to as 'administrative isolation' of inmates who are deemed at risk of escaping or disturbing prison order. The level of psychiatric morbidity among this group of inmates tends to be high — as is also the case with those subjected to isolation as punishment.¹⁴ Recent years have, in some jurisdictions, seen an increase in the use

of solitary confinement as an administrative tool. Probably the most well known example of this is the so-called Supermax prisons in the US. Conditions in these facilities typically include solitary confinement for 22.5 to 24 hours each day in a barren environment under constant high-tech surveillance with limited human contact, with exercise being carried out in isolation and without access to recreational equipment. Visits and phone calls are infrequent and severely restricted if allowed at all. Placement in Supermax

can be indeterminate and go on for years and even decades.¹⁵

Solitary confinement is sometimes also an integral part of regimes on 'death row'. Such arrangements have, for example, been reported by the CPT (The European Committee for the Prevention of Torture) during visits to Bulgaria and the Ukraine.¹⁶ In his latest report Manfred Nowak also described encountering death row prisoners in prolonged solitary confinement in Abkhazia and Mongolia.¹⁷

Political prisoners can also be subjected to solitary confinement. According to Human Rights Watch forty political prisoners in Tunisia in 2005 had been subjected to prolonged solitary confinement for several years and up to eleven years in one particular case.¹⁸

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11. 2008 General Assembly report — A/63/175, 'Interim report of the Special Rapporteur on torture and other cruel, inhuman, or degrading treatment or punishment', 28. July 2008, p. 19. f.
12. Hans Jørgen Engbo 'Straffuldbyrdsesret' 2. edition, Copenhagen 2005, p. 264.
13. 2008 General Assembly report — A/63/175, p. 19. NB:
14. Peter Scharff Smith (2006) 'The Effects of Solitary Confinement on Prison Inmates: A Brief History and Review of the Literature' in Michael Tonry (ed.) *Crime and Justice*, vol. 34, 2006, p. 451 ff.
15. Craig Haney (2003) 'Mental Health Issues in Long-Term Solitary and 'Supermax' Confinement' in *Crime & Delinquency* 49(1), 2003; Sharon Shalev (2005) *Solitary control and punitive isolation: 'new' forms of solitary confinement in Supermax prisons of the USA*, Phd. thesis, London 2005, p. 48; Lorna Rhodes (2004) *Total Confinement. Madness and Reason in the Maximum Security Prison*, 2004; Roy King (1999) 'The rise and rise of supermax: An American solution in search of a problem?' in *Punishment and Society* (1), 1999.
16. See Jim Murdoch (2006) *The treatment of prisoners. European standards*, Strasbourg 2006, p. 236 f.
17. 2008 General Assembly report — A/63/175, p. 19.
18. Human Rights Watch (2005) 'Tunisia. Crushing the person, crushing a movement. The solitary confinement of political prisoners', April 2005, vol. 17, No 4(E).

Another well known use of solitary confinement is pre-trial where isolation can be implemented for remand prisoners to protect an ongoing criminal investigation. This practice has been termed a particularly 'Scandinavian phenomenon'¹⁹, and Denmark, Norway, Sweden, and to some extent Iceland, have all received international human rights criticism on this account in the last decades.²⁰ In Denmark, for example, 8 per cent of all pre-trial detainees were subjected to solitary confinement during 2006, although an official large scale scientific study in 1997 recommended the abandonment of this practice on medical and psychological grounds.²¹

When used pre-trial, the impact of solitary confinement can pressure prisoners into confessing or giving evidence, even if this was not the intention. But solitary confinement and other isolation practices can also be used — and indeed are used — purposely as a part of coercive interrogation. This has been the case both at Guantanamo and in facilities in Iraq and Afghanistan, where isolation practices have often been used together with other coercive and abusive techniques.²² According to a report from Physicians for Human Rights, 'the use of isolation, sensory deprivation, and sensory overload became almost routine in the treatment of detainees'. One Iraqi detainee reported 'having been taken to a dark room where he was shackled for most of the time and kept isolated and naked in a cold cell for approximately the first three weeks of his incarceration there'.²³ Such practices were typically part of an interrogation process where the interrogators relied on the effects of solitary

confinement to induce compliance and assist with eliciting information from detainees.

Finally, a somewhat related use of solitary confinement from Communist Cold War detention and interrogation methods is the use of isolation in achieving 'thought reform' — popularly known as brain washing.²⁴ The UN Special Rapporteur has received allegations of prolonged solitary confinement at a Chinese re-education facility for women, where 'detainees alleged that they were held [in small solitary confinement cells] for up to 60 days, where they received 'training' to induce them to renounce their beliefs'.²⁵

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Developing standards — the period before WWII

The international standards that apply today with regard to prisons in general and solitary confinement in particular have largely been conceived within a human rights framework, though the process of creating such standards goes back to the period before the Second World War and the creation of the human rights conventions. Evidence on the detrimental health effects of Pennsylvania

model imprisonment continued to mount during the 19th and early 20th centuries and gradually influenced practice through developing national legal standards or international expert recommendations. As an example of the former, the U.S. Supreme Court ruled in 1890, in a case concerning solitary confinement of a prisoner under sentence of death in the state of Colorado, that solitary confinement 'was an additional punishment of the most important and painful character' and described vividly how solitary confinement in US nineteenth-century prisons had

19. Malcolm Evans and Rod Morgan describe pre-trial solitary confinement as a '... peculiarly Scandinavian phenomenon...' in *Preventing Torture*, 1998, p. 247.
20. Peter Scharff Smith (2005) 'Varetægtsfængsling i isolation – en besynderlig Skandinavisk tradition?' in *Social Kritik*, no. 99, June 2005. See also Peter Scharff Smith (2006) 'Prisons and Human Rights: The Case of Solitary Confinement in Denmark and the US from the 1820s until Today' in Stéphanie Lagoutte, Hans-Otto Sano and Peter Scharff Smith (eds.) *Human Rights in Turmoil. Facing Threats, Consolidating Achievements*, 2006, pp. 221-248.
21. See pre-trial solitary confinement figures here: <http://www.rigsadvokaten.dk/page62.aspx?recordid62=1122>. Concerning the 1997 report, see Henrik Steen Andersen, Tommy Lillebæk and Dorte Sestoft 'Efterundersøgelsen – en opfølgingsundersøgelse af danske varetægtsarrestanter', 1997, p. 59.
22. See for example: 'Broken Laws, Broken Lives. Medical Evidence of Torture by US Personnel and its Impact', a report by Physicians for Human Rights, June 2008, p. 77 ff.; 'Report on Torture and Cruel, Inhuman, and Degrading Treatment of Prisoners at Guantanamo Bay, Cuba', Center for Constitutional Rights, July 2006, p. 16. f.; Peter Scharff Smith (2006) 'Fra det moderne fængselsvæsen til Guantanamo. Isolationsfængsling, læger, fangebehandling og afhøringsmetoder 1830 til i dag' in *Bibliotek for Læger*, vol. 198, March 2006, p. 28 f.
23. 'Broken Laws, Broken Lives. Medical Evidence of Torture by US Personnel and its Impact', a report by Physicians for Human Rights, June 2008, p. 78.
24. Kathleen Taylor (2004) 'Brainwashing. The Science of Thought Control', Oxford University Press, 2004.
25. 2008 General Assembly report — A/63/175, p. 19.

affected the inmates and damaged their mental health severely.²⁶

This condemnation of solitary confinement was somewhat slower to develop within the international community of prison experts, largely due to the continuing popularity of the Pennsylvania model across several nations. Three international prison conferences were held between 1846 and 1857 during the heyday of the Pennsylvania model and although both this and the Auburn system were heavily debated, there was no general criticism of the role of solitary confinement. On the contrary the eight resolutions adopted at the Frankfurt conference in 1846 'approved the policy of separate confinement' where no communication was allowed between inmates.²⁷

The International Prison Commission (IPC) held further conferences from 1872 onwards. At the penitentiary congress in London in 1872, solitary confinement was subject to a lively discussion but no resolutions.²⁸ This undoubtedly reflected the fact that the Pennsylvania system was still practised in several countries, as illustrated by the minutes from the later conference in Brussels in 1900. Here the cellular system of isolation was endorsed although with less enthusiasm than previously. The Belgians even insisted 'that cellular imprisonment [i.e. solitary confinement] even prolonged ten years or beyond, assuming the previous or subsequent elimination of certain elements, has no more unfavorable effect upon the physical or mental health of prisoners than any other mode of imprisonment'.²⁹

At a penitentiary congress in Prague in 1930, however, international resolutions were drawn up that referred to some troubling aspects of solitary confinement. The congress specified that if solitary confinement was used for a short duration, adequate medical service was to be available. The congress furthermore advised that solitary confinement should not be used in connection with sentences of long duration.³⁰ Nine years later Wilson and Pescor concluded in their book *Problems in Prison Psychiatry*

that the inmates in Pennsylvania model prisons 'went insane instead of being reformed', and described the use of solitary confinement during both day and night as uncivilized.³¹

In 1935 a penitentiary congress was held in Berlin and the IPC was transformed into the IPPC (the International Penal and Penitentiary Commission). The Berlin conference was heavily influenced by the Nazi regime and Dr. Goebbels was among the official speakers.³² The IPPC continued to cooperate with the Nazis during the war and consequently it was dissolved by the UN General Assembly in 1950 and the international work with prison standards continued within the UN.

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Developing standards — Human rights and solitary confinement after WWII

It is not possible to provide a detailed account of all the relevant human rights instruments, principles, and judgments within the scope of the present article. So the following will only mention some of the important international and regional (European) instruments and the standards they promote.

On an international level there are several Conventions — as well as the original Declaration of Human Rights from 1948 — which are relevant to the area of prisons and human rights. But these Conventions do not themselves address the issue of solitary confinement directly although their articles of course form the basis of interpretations of its use. Most importantly, the UN Convention on Civil and Political Rights (ICCPR) from 1966 states that 'All persons deprived of their liberty shall be treated with humanity and with respect for the inherent dignity of the human person' (Article 10.1). The Human Rights Committee has later interpreted this in the following manner: '... persons deprived of their liberty [may not] be subjected to any hardship or constraint other than that resulting from the deprivation of liberty; respect for the dignity of such persons must be guaranteed under the same conditions as for that of free persons.'

26. In re Medley, 134 U.S. 160 [1890].

27. Benedict S. Alper and Jerry F. Boren (1972) 'Crime: International Agenda', 1972, p. 14.

28. Negley K. Teeters (1949) 'Deliberations of the International Penal and Penitentiary Congresses. Questions and Answers 1872-1935' Temple University Book Store 1949, p. 38.

29. Quoted from Alper and Boren (1972), p. 40.

30. Teeters (1949), pp. 38, 110, 172. See also Smith 2006, p. 467.

31. Wilson, J. G., and M. J. Pescor (1939) 'Problems in Prison Psychiatry', Idaho: Caldwell, 1939, p. 24.

32. Alper and Boren 1972, p. 71.

Persons deprived of their liberty enjoy all the rights set forth in the Covenant, subject to the restrictions that are unavoidable in a closed environment'.³³

But if we move from the actual human rights conventions to the area of soft law we find human rights instruments that refer more specifically to isolation practices. This was only indirectly the case with the so-called UN Standard Minimum Rules (SMR) for the Treatment of Prisoners that were adopted at the first UN congress on the prevention of crime and treatment of offenders in Geneva in 1955, where the UN in effect continued the work of the IPC/IPPC. But a subsequent revision in 1977 prohibits the use of close confinement and placement in dark cells specifically and states that 'Punishment by close confinement ... shall never be inflicted unless the medical officer has examined the prisoner and certified in writing that he is fit to sustain it' (Rule 32,1). In 1990 the so-called UN Basic Principles for the Treatment of Prisoners were more direct, stating that 'Efforts addressed to the abolition of solitary confinement as a punishment, or to the restriction of its use, should be undertaken and encouraged' (principle 7).

The UN Committee Against Torture (CAT) has criticized isolation practices in different parts of the world and has, for example, recommended that 'the use of solitary confinement be abolished, particularly during pre-trial detention, or at least that it should be strictly and specifically regulated by law (maximum duration, etc.) and that judicial supervision should be introduced'.³⁴ The UN committee on the Rights of the Child has further recommended that solitary confinement should not be used against children.³⁵

On a regional level the formation of the European Council in 1949 resulted in the creation of the European Convention on Human Rights from 1950 and the European Convention for the Prevention of Torture and Inhuman or Degrading Treatment or Punishment from 1987. The latter led to the creation of the European Committee for the Prevention of Torture (CPT), which has played a key role in promoting regional human rights standards in

relation to prisons. The CPT has stated that solitary confinement can amount to inhuman and degrading treatment and thereby violate the convention. The CPT has also, like the CAT, criticized isolation practices in several countries and recommended reform — that is either abolishing or limiting the use of solitary confinement to exceptional circumstances. The CPT has also on several occasions stressed the importance of securing isolated inmates a higher level of social contact.³⁶ Finally, the revised European Prison Rules of 2006 clearly state that 'Solitary confinement shall be imposed as a punishment only in exceptional cases and for a specified period of time, which shall be as short as possible' (rule 60.5).

But the above recommendations and standards that deal specifically with solitary confinement lie within the area of soft law, which is not in itself legally binding — although they may become so if they are applied in a court of law. In terms of actual case law there are relevant decisions involving isolation regimes from the UN Human Rights Committee as well as the European Court of Human Rights and the former Commission on Human Rights.

The UN Human Rights Committee has stipulated that use of prolonged solitary confinement may amount to a breach of Article 7 (which prohibits torture, cruel, inhuman or degrading treatment or punishment) of the ICCPR³⁷ and has in the case of *Campos v. Peru* found a specific isolation regime to violate both Article 7 and Article 10 (the right to be treated with humanity and dignity) of the ICCPR.³⁸

The European Court of Human Rights (ECtHR) has through numerous judgments been instrumental in securing prisoner rights in many areas, and has considered several cases involving the issue of solitary confinement in prisons. It is beyond the scope of the present article to provide a detailed account of these interesting cases. One can, however, conclude that while it has on the one hand been recognized that '...complete sensory isolation coupled with total isolation, can destroy the personality and constitutes a form of inhuman treatment which cannot be justified by the requirements of security or any other reason'³⁹,

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33. The Human Rights Committee, General Comment No. 21[44], article 10 (1-3) 1992.

34. CAT, Visit report, Denmark, 1. May 1997, para 186.

35. CRC/C/15/Add.273, 'Denmark', 30 September 2005, para. 58 a.

36. See footnote 1.

37. Human Rights Committee, General comment 20/44, 3. April 1992.

38. *Campos v. Peru* 9. January 1998.

39. *Ramirez Sanchez v. France*, Grand Chamber, 4. July 2006, para. 123.

the Court has on the other hand been very reluctant to judge actual solitary confinement regimes as a violation of the Convention. This was clearly demonstrated in the *Rohde vs. Denmark* case where a detainee who became insane as a result of a nearly one year stay in pre-trial solitary confinement. The Danish Supreme Court and the Danish Legal-Psychiatric Clinic acknowledged this fact, which was not disputed by the ECtHR. Still the court found no violation of Article 3 of the ECHR — albeit three out of seven judges gave dissenting opinions and actually did find such a breach.⁴⁰

Such judgments have caused experts to argue that courts sometimes are reluctant to either acknowledge or deal with the psychological pains of imprisonment — in this case specifically the effects of solitary confinement. The ECtHR has specifically — and in my mind quite correctly — been criticised for ‘... a lack of imagination, or at least of judicial understanding of the impact of solitary confinement upon prisoners and too-ready an acceptance of state interests’.⁴¹ In the case of *Rhode v. Denmark* the Court even seemed to transfer some of the responsibility for monitoring the health of the applicant to members of his family, who — according to the Court — should have reported their concerns about the mental state of the applicant to the prison management and prison doctors.⁴² On this issue the three dissenting judges noted that the responsibility for effectively monitoring the applicant’s health and carrying out a timely psychological or psychiatric assessment (which was only done once in Rhode’s case and less than one month after he had been detained in solitary confinement) ‘should not depend on a request [by relatives] but instead should be part of a system of automatic regular monitoring of long-term detainees in solitary confinement. We believe that only under

these conditions can the required monitoring be considered effective’.⁴³

I think that the ECtHR has been much more progressive in terms of securing prisoners rights in many other cases — also specifically with regard to the state’s responsibility for monitoring the health of inmates.⁴⁴ But with regard to the use of solitary confinement the question still remains: how can we with the current standards and case law protect the rights and health of those subjected to it?

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Developing standards in light of the health effects of solitary confinement

As we have seen, human rights conventions and soft law set a certain standard with regard to the use of solitary confinement. But these standards are clearly minimum standards and one can argue that they are not in line with what research tells us about the actual effects of solitary confinement. To put it differently: by following human rights standards with regard to solitary confinement a state can avoid the worst kinds of abuses in this area — but can still maintain practices which, according to research and experience, severely endanger the health of those subjected to it.

I would argue that we are not only faced with the challenge of promoting existing human rights standards with regard to solitary confinement — although this of course still makes very good sense — but we also need to work on the further development of the standards themselves. Research has made it increasingly clear that policy and international standards in this area are out of step with the empirical findings.

Whilst international debate over the effects of solitary confinement seems to have been resolved around the 1930s in a broad consensus between psychiatrists and prison experts that it was detrimental⁴⁵ — discussion was reopened after WWII in

40. *Rhode v. Denmark*, Application no. 69332/01, 21. July 2005.

41. Jim Murdoch ‘The treatment of prisoners. European standards’ Council of Europe Publishing 2006, p. 255.

42. *Rhode v. Denmark*, Application no. 69332/01, 21. July 2005, para. 109.

43. *Rhode v. Denmark*, Application no. 69332/01, 21. July 2005, see ‘JOINT DISSENTING OPINION OF JUDGES ROZAKIS, LOUCAIDES AND TULKENS’.

44. See *KEENAN v. THE UNITED KINGDOM*, Application no. 27229/95, 3. April 2001.

45. Peter Scharff Smith (2006) ‘The Effects of Solitary Confinement on Prison Inmates: A Brief History and Review of the Literature’ in Michael Tonry (ed.) *Crime and Justice*, vol. 34, 2006, p. 466 f.

connection with differing opinions regarding the experimental psychological research into sensory deprivation.⁴⁶ In recent decades however, the vast majority of studies on the effects of solitary confinement in places like Norway, Denmark, Switzerland, South Africa, Canada, and USA have reported that solitary confinement has negative health effects, and a broad consensus that the health effects clearly disfavour the use of solitary confinement has been re-established.⁴⁷ The central harmful feature is that it reduces meaningful social contact to an absolute minimum: a level of social and psychological stimulus that many individuals will experience as insufficient to remain reasonably healthy and relatively well functioning' (for further information see Craig Haney's article in the current issue).⁴⁸

Unfortunately, the human rights minimum standards currently in operation in this area do not secure a sufficient amount of meaningful social contact to mitigate its harmful effects. Current standards protect isolated individuals from severe abuse in the form of, for example, strict sensory deprivation or clearly unfit physical conditions, but regimes in which prisoners' physical conditions live up to normal cell standards and in which they receive an hour exercise in the open air each day cannot currently be considered a violation of human rights – depending on the exact circumstances of the regime. Some close confinement regimes may be criticised by the CPT, CAT, and others on the basis of soft law standards but this does not mean that a court of law — such as the ECtHR — will judge them to be in violation of actual conventions. Not even prolonged solitary confinement will necessarily amount to a human rights violation⁴⁹ — regardless of the fact that research suggests that many of these

individuals will suffer serious detrimental health effects.

We therefore have to improve human rights standards in this area — in particular with regard to the fact that isolated inmates need access to psychologically meaningful social contact. The creation of the so-called *Istanbul Statement on the Use and Effects of Solitary Confinement* constitutes such an

attempt. This statement was adopted on 9. December 2007 at the International Psychological Trauma Symposium in Istanbul and is signed by several prominent international experts in the field of solitary confinement, prisons and torture representing numerous national and international institutions.⁵⁰

The Istanbul Statement determines that the use of solitary confinement should be absolutely prohibited for 'death row and life-sentenced prisoners by virtue of their sentence', for 'mentally ill prisoners', and for 'children under the age of 18'.⁵¹ The Statement furthermore concludes that 'regardless of the specific circumstances, and whether solitary confinement is used in connection with disciplinary or administrative segregation or to prevent collusion in remand prisons, effort is required to raise the level

of meaningful social contacts for prisoners. This can be done in a number of ways, such as raising the level of staff-prisoner contact, allowing access to social activities with other prisoners, allowing more visits, and allowing and arranging in-depth talks with psychologists, psychiatrists, religious prison personnel, and volunteers from the local community. Especially important are the possibilities for maintaining and developing relations with the outside world including spouses, partners, children, other family and friends'.⁵²

Such a recommendation will in the long run hopefully be translated into specific standards and

In recent decades
the majority of
studies on the
effects of solitary
confinement in
places like Norway,
Denmark,
Switzerland, South
Africa, Canada, and
USA have reported
that solitary
confinement has
negative health
effects ...

46. See, for example, Morris and Kurki (2001), p. 113, and Michael Jackson. *Justice Behind the Walls. Human Rights in Canadian Prisons*, available online at <http://www.justicebehindthewalls.net/>. See also Smith 2006.

47. Haney 2006; Smith 2006; Henrik Steen Andersen 2006 'Mental Health in Prison Populations. A review – with special emphasis on a study of Danish prisoners on remand' in *Acta Psychiatrica Scandinavica Supplementum*, 110(424), 2004.

48. Smith (2006), p. 503.

49. See for example *Rohde vs. Denmark*, *Application no. 69332/01*, 21. July 2005, and *Sanchez vs. France*, *Application no. 59450/00*, 27. January 2005.

50. See footnote 1.

51. See footnote 1.

52. See footnote 1.

practices. And while it 'is of course difficult — and most likely impossible — to identify some sort of breaking point: *the* minimum level of social contact needed to avoid the effects of solitary confinement ... it seems fair to conclude that one or two hours of out of cell time (even including some social contact) is not enough. Whether three, four, five, or six hours would suffice to eliminate the differences between isolation and non-isolation is not known, but even three hours of out of cell time including social contact would most likely create much better and mentally healthier conditions'.⁵³

It is very encouraging to note that the UN Special Rapporteur on Torture in his latest report to the General Assembly also pushed for better standards with regard to the use of solitary confinement. In the opinion of the Special Rapporteur 'the use of solitary

confinement should be kept to a minimum, used in very exceptional cases, for as short a time as possible, and only as a last resort. Regardless of the specific circumstances of its use, effort is required to raise the level of social contacts for prisoners: prisoner-prison staff contact, allowing access to social activities with other prisoners, allowing more visits and providing access to mental health services'.⁵⁴ Finally the Special Rapporteur 'strongly encourages States to reflect upon the [Istanbul] Statement as a useful tool in efforts to promote the respect and protection of the rights of detainees'.⁵⁵ With such a clear recommendation to the UN — and thereby States across the globe — from the highest UN expert in the field of torture we can hopefully expect better protection for isolated prisoners in the future.



PERRIE LECTURES ESSAY AWARD 2009



The Perrie Lectures is an annual event which has the purpose of stimulating dialogue between criminal justice organisations, the voluntary sector and all those with an academic, legal or practical interest in offenders and their families. The Perrie Lecture committee share the belief that through these lectures and the ensuing dialogue, the care of offenders can be improved and penal policy, in its broadest sense, advanced.

To supplement the lectures the committee has decided to initiate an essay competition for NOMS staff.

The aim of this competition is to attract views on the subject of the lectures from a variety of perspectives within the NOMS agency, this may be at frontline staff level in prisons or the community or from a management and strategic level as well as from administrators. In so doing we hope that we can spread the debate around the subject of the lectures and so further the aims and purpose of the Perrie Lectures.

The title for the lectures in 2009 is

'WHAT ARE PRISONS FOR? TO HOLD, TO HELP OR TO HURT?'

Essays are therefore invited on this year's title above.

With essay length to be **1000 – 2000 words.**

All essays will be reviewed by the committee and a winner announced in time to attend the lectures and receive their **£250 award.**

Essays must be submitted to Al Reid, al.reid@hmps.gsi.gov.uk or in hard copy to Rm 105, Cleland House, Page Street, London SW1P 4LN

Closing date for entries is Friday 17th April 2009

**The 2009 Perrie Lectures will take place on 5th June 2009 at
HM Prison Service College, Newbold Revel, Rugby.**

53. Smith 2006, p. 505.

54. 2008 General Assembly report — A/63/175, p. 21.

55. 2008 General Assembly report — A/63/175, p. 21.